

Department of Pediatric Surgery
Discharge Summary

CR No.: anonymous	Name: anonymous	Age/Gender: 7.9 Yr/M
Admission No.: anonymous	Department Unit: Pediatric Surgery (Ped Sx Unit 1)	Consultant/Faculty: anonymous
Ward: Pedia Sx 3 C 4	Room/Bed: 3 C 4 Ward/ 3 C 4 Pae4	Patient Category: Ayushman
Discharge Type: Normal Discharge	Father/Spouse Name: anonymous	Occupation:
Contact No.: anonymous	Address: anonymous	State/Country India
D.O.A. : 26-Mar-2023 17:54	D.O.D. : 04-Apr-2023	D.O. Print Report: 08-Feb-2026 15:28

FINAL DIAGNOSIS:

HIRSCHPRUNG DISEASE

OPERATION PERFORMED:

TRANSVERSE COLOSTOMY WITH RECTAL BIOPSY On 28-Mar-2023

OPERATIVE FINDINGS: OT NOTES:

OPERATION PERFORMED: RIGHT TRANSVERSE COLOSTOMY RECTAL BIOPSY

SURGEON: DR SONALI ASSISTANT: DR REVANTH ANAESTHESIA: DR MUSSAVIR

PROCEDURE:

UNDER AAP, IN SUPINE POSITION, RIGHT UPPER TRANSVERSE INCISION GIVEN AND INCISION DEEPENED BY LAYERS AND ENTERED INTO PERITONEAL CAVITY. RIGHT TRANSVERSE COLON IDENTIFIED AND BROUGHT OUT AS STOMA. FASCIAL FIXATION DONE WITH 3-0 PDS. SPUR CONSTRUCTION DONE AND STOMA MATURATION DONE WITH 4-0 PDS. PATIENT SHIFTED TO LITHOTOMY POSITION. FULL THICKNESS RECTAL BIOPSY TAKEN AND MUCOSAL DEFECT CLOSED WITH 2-0 PDS. HEMOSTASIS ACHIEVED. RECTAL PACK KEPT.

CHIEF COMPLAINTS:

ABDOMINAL DISTENTION

CONSTIPATION

HISTORY OF PRESENT ILLNESS: ACCORDING TO PATIENT ATTENDER, PATIENT HAS H/O PASSING DELAYED MECONIUM AFTER BIRTH. HE HAS H/O OF CHRONIC CONSTIPATION RELIEVED WITH ENEMA SINCE BIRTH A/W ABDOMINAL DISTENTION SINCE BIRTH.

EXAMINATION FINDINGS :

GENERAL:

PR: 95/M TEMP: 98.2F

RR: 24/MWEIGHT : 12.2 KG

No Pallor, No Cyanosis, No Clubbing, No Generalised LNE, No Generalised Edema

CNS: WNL

CVS: S1S2 HEARD

RS: BL AIRENTRY PRESENT

PA: DISTENDED, NO TENDERNESS

PR FECAL STAIN PRESENT



INVESTIGATION DETAILS:

Requisition Date	Test Name	Parameter Name	Result	Ref Range & Unit
26-Mar-2023 18:12	BLOOD GROUPING	Blood Group	B	-
	ABO and RhD			
		Note	Refer Reports*	-
		RH Type	Positive	-

***Refer Lab Reports for details.**

OTHER INVESTIGATIONS: 1.TAB CEFIXIME 80 MG PO BD X2 DAYS

2.TAB PCM 250MG PO SOS

3.KEEP LOCAL AREA CLEAN AND MOIST

4.CONTINUE STOMA CARE

5. APPLY ZINC OXIDE CREAM AROUND THE SURROUNDING SKIN OF STOMA

HOSPITAL COURSE: PT ADMITTED WITH FEATURES SUGGESTIVE OF HIRSCHPRUNG'S DISEASE. HE UNDERWENT RIGHT TRANSVERSE COLOSTOMY WITH RECTAL BIOPSY ON 28/3/23. POST OPERATIVE PERIOD WAS UNEVENTFUL. STOMA HEALTHY AND FUNCTIONAL. BIOPSY REPORT AWAITED. PATIENT IS HENCE BEING DISCHARGED WITH FOLLOWING ADVICE AND ASKED TO FOLLOW UP IN PAEDIATRIC SURGERY OPD WITH WITH BIOPSY REPORT

CONDITION AT DISCHARGE: PATIENT IS HEMODYNAMICALLY STABLE. PASSED URINE STOOLS. TOLERATING ORALS. STOMA HEALTHY, FUNCTIONAL.

DISCHARGE BY : anonymous

DISCHARGE REMARKS :

FOLLOW UP ON : 11-Apr-2023

FOLLOW UP CONSULTANT / FACULTY : anonymous

FOLLOW UP ADVICE :

Summary Prepared By

anonymous

Summary Approved By

anonymous